

C803

Data Analytics and Information Governance

Armored Drill

8 concepts · 12 items · 4 transfer items

What this is. Every concept in this course's public description, re-asked with more disguise than a practice test uses. Each stem carries facts that do not decide it. Every wrong answer is a named trap.

Source. Scope from the course's public description in the WGU Institutional Catalog (September 2026); items written from the field's standard body of knowledge. No WGU assessment item, course material or proprietary content is used or reproduced. Not affiliated with or endorsed by Western Governors University.

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How to run it

Paper first. Pencil. One pass, no notes, a timer if you want the pressure.

1. **Before you read the options**, fill the strip under the item: the target word (what is actually being asked), and the one fact in the stem that decides it.
2. **Principle before arithmetic**. Name the rule, then compute.
3. **Camouflage**. Most of every stem does not matter. Ask: which single fact decides this?
4. **Inverse pairs are the bait**. If two options are opposites, one of them is the trap built for you.
5. **Mark sure, unsure or guess** honestly. Sure-and-wrong is the most valuable miss on the page — it is a wrong mental model.
6. **Absolutes** (always, never, guarantee) are usually wrong. And recognizing an answer is not the same as owning it.
7. Items with a **heavy black bar** on the left are **transfer items**: the same principle, reversed or solved for a different variable.
8. Stop at the STOP page. Then grade, fill the triage grid, and tally the traps you fell for. The biggest tally is what you drill next.

The strip, filled in, looks like this:

Target word _____	The one fact that decides _____	Answer _____	[] sure [] unsure [] guess
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Items

1.

A health system with 9 hospitals makes a nurse manager responsible for defining and monitoring the quality of its fall-risk score field, while a committee sets enterprise-wide data policy. The system recently upgraded its email platform. What is the nurse manager's role?

- A. Governance committee member.
- B. Email system administrator.
- C. Data steward.
- D. Database administrator.

Target word	The one fact that decides	Answer	[] sure [] unsure [] guess
_____	_____	___	

2.

An analyst at a hospital with a new rehab gym finds ages of 212 and discharge dates earlier than admission dates in an extract of 50,000 records. Which control would have stopped these errors at entry?

- A. An annual data quality audit.
- B. Range and logic edit checks in the entry screens.
- C. Encrypting the database.
- D. Pulling a larger extract than 50,000 records.

Target word	The one fact that decides	Answer	[] sure [] unsure [] guess
_____	_____	___	

3.

A population health team at a regional accountable care organization with a new logo builds a model that estimates each patient's probability of an ED visit in the next 90 days. Which type of analytics is this?

- A. Descriptive.
- B. Diagnostic.
- C. Predictive.
- D. Prescriptive.

Target word	The one fact that decides	Answer	[] sure [] unsure [] guess
_____	_____	___	

4. [TRANSFER — reversed stem / different variable]

A dashboard recommends the exact staffing level for each unit tomorrow that meets ratios with the least overtime. Which type of analytics is this?

- A. Diagnostic.
- B. Predictive.
- C. Descriptive.
- D. Prescriptive.

Target word	The one fact that decides	Answer	[] sure [] unsure [] guess
_____	_____	___	

5.

A hospital with a rooftop helipad tracks the percentage of heart failure patients who receive discharge education before leaving. Which Donabedian category is this measure?

- A. Structure.
- B. Outcome, because every quality measure tracks results.
- C. Process.
- D. Outcome.

Target word	The one fact that decides	Answer	[] sure [] unsure [] guess
_____	_____	___	

6. [TRANSFER — reversed stem / different variable]

Which is an outcome measure?

- A. 30-day mortality after a heart attack.
- B. Number of certified cardiac nurses on staff.
- C. All of these, since each affects patient results.
- D. Percentage of heart attack patients given aspirin on arrival.

Target word	The one fact that decides	Answer	[] sure [] unsure [] guess
_____	_____	___	

7.

An ICU at a teaching hospital known for its trauma program recorded 6 central line-associated bloodstream infections over 4,000 central-line days and 1,200 admissions. What is the infection rate per 1,000 central-line days?

- A. 0.15
- B. 5.0
- C. 666.7
- D. 1.5

Target word	The one fact that decides	Answer	[] sure [] unsure [] guess
_____	_____	___	

8. [TRANSFER — reversed stem / different variable]

A unit's pressure injury rate is 2.0 per 1,000 patient days, and it had 5,000 patient days. How many pressure injuries occurred?

- A.** 10
- B.** 2,500
- C.** 2
- D.** 10,000

Target word	The one fact that decides	Answer	[] sure [] unsure [] guess
_____	_____	___	

9.

A clinic that recently switched to a cloud EHR has boxes of paper records past their state-required retention period, and none are under a legal hold. The office manager plans to put them in the regular recycling bin tonight. What is the correct disposition?

- A.** Recycle them as planned.
- B.** Scan them, then recycle the paper.
- C.** Keep them forever to be safe.
- D.** Destroy them securely, such as by shredding, and keep a record of destruction.

Target word	The one fact that decides	Answer	[] sure [] unsure [] guess
_____	_____	___	

10.

Hospital A, a 500-bed referral center with a new parking garage, has a raw mortality rate of 4%. Hospital B, a small community hospital, has 2%. Hospital A treats far sicker patients. What is the best way to compare them?

- A.** Conclude Hospital B is better because 2% is lower than 4%.
- B.** Compare risk-adjusted rates, such as observed-to-expected ratios.
- C.** Conclude Hospital A is worse because it is larger.
- D.** Compare raw death counts instead of rates.

Target word	The one fact that decides	Answer	[] sure [] unsure [] guess
_____	_____	___	

11. [TRANSFER — reversed stem / different variable]

A hospital's observed-to-expected mortality ratio is 0.8. What does this mean?

- A.** Fewer deaths occurred than expected for its patient mix.
- B.** 80% of patients survived.
- C.** The mortality rate is 0.8%.
- D.** 20% more deaths occurred than expected.

Target word	The one fact that decides	Answer	[] sure [] unsure [] guess
_____	_____	___	

12.

A hospital that recently won a design award must collect core inpatient data elements, such as principal diagnosis and discharge disposition, with consistent definitions for every inpatient discharge. Which data set defines these?

- A.** OASIS.
- B.** UHDDS.
- C.** HEDIS.
- D.** MDS.

Target word	The one fact that decides	Answer	[] sure [] unsure [] guess
_____	_____	____	

STOP

Answers and dissection follow. Finish every item and every strip first.

Key and dissection

Struck-through text is the camouflage: facts that do not decide the item.

1. Answer C Concept 1

A health system ~~with 9 hospitals~~ makes a nurse manager responsible for defining and monitoring the quality of its fall-risk score field, while a committee sets enterprise-wide data policy. ~~The system recently upgraded its email platform.~~ What is the nurse manager's role?

Target word: role **Decides:** Responsible for one element's definition and quality.

A	Adjacent category	The committee sets policy; the manager carries it out for one field.
B	Camouflage	The email upgrade does not decide the data role.
C	Correct	Stewards own the definition and quality of specific data.
D	Confusion set	A technical custodian who runs the systems.

2. Answer B Concept 2

An analyst at a hospital ~~with a new rehab gym~~ finds ages of 212 and discharge dates earlier than admission dates in an extract of 50,000 records. Which control would have stopped these errors at entry?

Target word: preventive control **Decides:** Impossible values and illogical dates.

A	Inverted operation	Finds errors after the fact rather than preventing them.
B	Correct	Validation rejects impossible values before they are saved.
C	Confusion set	Encryption protects confidentiality, not accuracy.
D	Wrong stem number	More records do not remove bad ones.

3. Answer C Concept 3

A population health team at a regional accountable care organization ~~with a new logo~~ builds a model that estimates each patient's probability of an ED visit in the next 90 days. Which type of analytics is this?

Target word: analytics type **Decides:** Estimating a future likelihood.

A	Confusion set	Summarizes what already happened.
B	Confusion set	Explains why something happened.
C	Correct	Estimates what is likely to happen.
D	Adjacent category	Recommends an action; this only estimates risk.

4. Answer D Concept 3 · transfer

A dashboard recommends the exact staffing level for each unit tomorrow that meets ratios with the least overtime. Which type of analytics is this?

Target word: reverse: recommendation **Decides:** It says what to do.

A	Confusion set	Explains causes of past results.
B	Adjacent category	Would forecast demand without choosing an action.
C	Dropped element	Drops the forward-looking recommendation.
D	Correct	Recommends the best action.

5. Answer C Concept 4

A hospital ~~with a rooftop helipad~~ tracks the percentage of heart failure patients who receive discharge education before leaving. Which Donabedian category is this measure?

Target word: measure category **Decides:** Whether a care step was delivered.

A	Adjacent category	Structure covers resources such as staff and equipment.
B	Absolute language	Many measures track processes or structures.
C	Correct	It measures care delivered.
D	Confusion set	Outcomes are results such as readmission or death.

6. Answer A Concept 4 • transfer

Which is an outcome measure?

Target word: reverse: find the outcome **Decides:** What happened to the patient.

A	Correct	A patient result.
B	Adjacent category	A structure measure.
C	Absolute language	Affecting outcomes does not make a measure an outcome.
D	Confusion set	A process measure.

7. Answer D Concept 5

An ICU at a teaching hospital ~~known for its trauma program~~ recorded 6 central line-associated bloodstream infections over 4,000 central-line days and 1,200 admissions. What is the infection rate per 1,000 central-line days?

Target word: device-day rate **Decides:** $6 \div 4,000 \times 1,000$.

A	Adjacent category	Scales per 100 instead of per 1,000.
B	Wrong stem number	Uses 1,200 admissions as the denominator.
C	Inverted operation	Divides line days by infections.
D	Correct	$6 / 4,000 \times 1,000 = 1.5$.

8. Answer A Concept 5 • transfer

A unit's pressure injury rate is 2.0 per 1,000 patient days, and it had 5,000 patient days. How many pressure injuries occurred?

Target word: solve for events **Decides:** Events = rate $\times$ days $\div$ 1,000.

A	Correct	$2.0 \times 5,000 / 1,000 = 10$.
B	Inverted operation	Divides days by the rate.
C	Wrong stem number	The rate, not the count.
D	Dropped element	Drops the division by 1,000.

9. Answer D Concept 6

A clinic ~~that recently switched to a cloud EHR~~ has boxes of paper records past their state-required retention period, and none are under a legal hold. The office manager plans to put them in the regular recycling bin tonight. What is the correct disposition?

Target word: disposition method **Decides:** PHI must be destroyed securely and documented.

A	Dropped element	Drops secure destruction; recycling exposes PHI.
B	Adjacent category	Scanning does not make unsecured disposal of the originals acceptable.
C	Absolute language	Over-retention adds cost and breach risk.
D	Correct	Secure, documented destruction protects PHI.

10. Answer B Concept 7

Hospital A, a 500-bed referral center ~~with a new parking garage~~, has a raw mortality rate of 4%. Hospital B, a small community hospital, has 2%. Hospital A treats far sicker patients. What is the best way to compare them?

Target word: fair comparison **Decides:** Patient mix differs sharply.

A	Dropped element	Drops the difference in patient severity.
B	Correct	Adjusting for severity makes the comparison fair.
C	Camouflage	Size alone does not decide quality.
D	Inverted operation	Counts ignore both size and severity.

11. Answer A Concept 7 • transfer

A hospital's observed-to-expected mortality ratio is 0.8. What does this mean?

Target word: interpret O/E **Decides:** Below 1.0.

A	Correct	Observed is 80% of expected.
B	Confusion set	The ratio compares observed to expected; it is not a survival rate.
C	Adjacent category	A ratio is not a rate.
D	Inverted operation	Above 1.0 would mean more than expected.

12. Answer B Concept 8

A hospital ~~that recently won a design award~~ must collect core inpatient data elements, such as principal diagnosis and discharge disposition, with consistent definitions for every inpatient discharge. Which data set defines these?

Target word: data set **Decides:** Inpatient discharge data.

A	Adjacent category	Used in home health.
B	Correct	The Uniform Hospital Discharge Data Set defines core inpatient elements.
C	Confusion set	Measures health plan performance.
D	Adjacent category	Used in long-term care facilities.

Triage grid

Write each item number in the box it belongs to.

	Right	Wrong
Sure	Owned.	Concept inversion — top priority. A wrong mental model; it survives rewording.
Unsure / guess	Recognition, not mastery. Re-drill with the transfer.	Not learned yet.

Trap tally

For every miss, find the letter you chose in the key and tally its trap. The biggest number is the pattern to drill.

Trap	What it does	In this drill	Your misses
Dropped element	Leaves out a required term or step.	4	
Wrong stem number	A figure copied from the prompt that is not the answer.	3	
Adjacent category	The right idea in the wrong bucket.	10	
Inverted operation	Direction flipped: add/subtract, cause/effect, higher/lower.	5	
Absolute language	Always, never, guarantee — where judgment applies.	3	
Confusion set	Every option is real; the test is exact role-matching.	9	
Camouflage	Answers a stem fact that was never the question.	2	

Anchor rows — the concept list

One row per concept: the rule to lock in, the trap that catches people, and the tell in the question that signals it.

#	Concept	Rule	Trap	Tell
1	Governance roles	A governance body sets enterprise data policy; data stewards define and monitor specific data elements; custodians such as DBAs run the technology.	Calling the steward a DBA or the policy committee.	A named person responsible for one field's definition and quality.
2	Preventing bad data at entry	Edit checks such as range and logic validations stop impossible values when data are entered.	Relying on after-the-fact audits alone.	Impossible ages or dates out of order.
3	Types of analytics	Descriptive: what happened; diagnostic: why; predictive: what is likely; prescriptive: what to do.	Calling a risk score prescriptive.	Probability of a future event vs. a recommended action.
4	Donabedian quality measures	Structure = resources and setting; process = care delivered; outcome = the result for the patient.	Calling any percentage an outcome.	Did the care happen, or what happened to the patient?
5	Healthcare rates	Rate = events ÷ correct denominator × scaling constant, such as infections per 1,000 device days.	Using the wrong denominator or constant.	Several possible denominators in one stem.
6	Retention and secure disposition	After retention ends and with no legal hold, destroy records securely (shred or pulverize) and document the destruction.	Recycling or keeping records forever.	Records past their retention date.
7	Benchmarking with risk adjustment	Compare providers with risk-adjusted rates or observed-to-expected ratios; below 1.0 is better than expected.	Ranking by raw rates when patient mix differs.	One provider treats much sicker patients.
8	Standard data sets	UHDDS defines core inpatient discharge data; HEDIS measures health plan performance; MDS covers long-term care; OASIS covers home health.	Mixing up which data set fits which setting.	The care setting named in the stem.

Course scope (public description): Data Analytics and Information Governance explores the structure, methods, and approaches for using health information in the healthcare industry. By focusing on quality data collection, analytics, and industry regulations, students will examine tools that ensure quality data collection as well as to use data to improve quality of care. This course has no prerequisites.